

Post-Thrombolytic Monitoring & Supportive Care (Alteplase or Tenecteplase)

These recommendations reflect widely accepted post-thrombolytic care guidelines but may be tailored based on institutional protocols and provider discretion.

Time-Sensitive Monitoring (First 24 Hours)

Time Interval	Neuro & Vital Checks
First 2 hours	q15 minutes
Next 6 hours	q30 minutes
Remaining 16 H	q1 hour

Labs & Imaging

- **Baseline before thrombolytic** (if not already drawn):
 - CBC, PT/INR, aPTT, BMP, glucose
- **Follow-up CT head at 24 hours post-dose or sooner if deterioration**
- Repeat labs if signs of bleeding or clinical changes

Blood Pressure Management

- **Goal BP post-thrombolytic:**
 - **<180/105 mmHg**
- **Avoid hypotension** (SBP <110 mmHg) — may worsen perfusion

- **Monitoring for Hemorrhage**

- *Watch for:*
 - Sudden **neurologic decline**
 - Severe **headache**
 - **Hypertension**
 - **Nausea/vomiting**
 - Hematuria, hematemesis, bruising
- **❓ If ICH suspected:**
 - Stop all antithrombotic
 - Urgent **head CT**
 - Type & screen, labs
 - Reverse with cryoprecipitate, TXA, aminocaproic acid (consult neurology/neurosurgery)

- **Hold the Following for 24 Hours**

- **Anticoagulants**
- **Antiplatelets**
- **Invasive procedures:** foley, central lines, arterial sticks

- **Supportive Care**

- Maintain **normoglycemia** (avoid hypo- and hyperglycemia)
- Treat **fever** aggressively (Tylenol preferred)
- Monitor for **DVT** – SCDs until safe for anticoagulation
- Evaluate for **dysphagia** before oral intake
- Head of bed elevated 30° unless contraindicated

Management of Cerebral Edema

For patients with large infarcts (especially MCA territory) or signs of herniation:

1. Head Positioning

- Elevate HOB to 30° unless contraindicated

2. Hyperosmolar Therapy

Agent	Dose
3% Saline	250 mL bolus over 20 min or continuous infusion
23.4% Saline	30 mL IV bolus over 10–15 min (central line)
Mannitol	0.25–1 g/kg IV over 20 min, repeat q6h PRN

- Monitor sodium, osmolality, renal function
- Avoid serum sodium >160 or osmolality >320 mOsm/kg

3. Other Measures

- **Intubation and sedation** for airway control or ICP control
- **Avoid hypotonic fluids**
- **Normothermia:** treat fevers aggressively
- **Consider neurosurgical decompression** for large infarcts with mass effect

- **Other Considerations**

- ICU or stroke unit admission for first 24h
- Blood glucose: maintain 140–180 mg/dL
- DVT prophylaxis: SCDs until anticoagulation safe
- Evaluate for dysphagia before PO
- ICU or stroke unit for continued close monitoring
- Multidisciplinary collaboration: neuro, nursing, pharmacy, rehab
- Document **exact time of thrombolytic bolus**
- Maintain IV access (2 lines recommended)
- Transition to secondary stroke prevention plan after 24h CT clear

